NURSING CARE OF A FAMILY WHEN A CHILD HAS A LONG-TERM OR TERMINAL ILLNESS

CHAPTER 56

OBJECTIVES

- Assess the adjustment of a child or family to a long-term or terminal illness.
- Formulate nursing diagnoses for a child with a long-term or terminal illness to help manage seamless transitions across differing healthcare settings.
- Identify expected outcomes for a child with a long-term or terminal illness.
- Describe common concerns of parents of children with a long-term or terminal illness.
- Implement nursing care for a child with a long-term or terminal illness, such as helping parents with time management and identification of resources.
- Evaluate effectiveness and achievement of expected outcomes for a child with a long-term or terminal illness and their families.
- Integrate knowledge of long-term or terminal illness with the interplay of the nursing process, the six competencies of QSEN, and family nursing to promote quality maternal and child health nursing care.

OVERVIEW

- An acute exacerbation of a chronic illness in children can bring fear and uncertainty to the child and their parents.
- The sudden onset and severity of symptoms after a period of remission can be a source of anxiety for families.
- Despite the resilience these families develop, they still require adequate support to adjust to the strain of disrupted daily routines, hospital visits, and home care.
- When a child's illness becomes long term or is one that is projected to have a terminal outcome, a family's capacity to cope can become stretched beyond its limits.
- Support is essential for such a family if it is to survive under this level of pressure and stress
- Quality of life in children with long-term or terminal illness is important to the overall health of children

ASSESSMENT

- A family's ability to cope is best assessed gradually and over a period of many interactions.
- Observing children in familiar settings such as the home, where they are most comfortable, or at school in a familiar atmosphere often reveals a great deal more about a child's present disease status or coping potential than does a formal test situation.
- Children with a long-term illness go through multiple tests and procedures in the process of diagnosis and follow-up care.
- As a result of this frequent interaction with healthcare providers, they may associate healthcare providers with pain and bodily harm rather than as those from whom they wish to receive care.
- Changing their perception may require maintaining a reassuring, gentle manner during assessment and subsequent interactions.

NURSING DIAGNOSIS

- Some examples of nursing diagnoses when a long-term illness is present include:
  - Impaired family processes related to recent diagnosis of chronic illness in the child
  - Impaired family coping related to the child's disability
  - Altered family coping related to the parents' inability to accept the child's long-term illness
  - Anticipatory grieving related to chronicity of the child's illness
- New issues develop when the child's disorder becomes terminal. The family must learn to accept not only the child's illness butalso its eventual outcome. Some examples of nursing diagnoses when a terminal illness is present include:
  - Hopelessness related to steady progression of the child's disease
  - Anticipatory grieving related to the child's terminal illness
  - Powerlessness related to inability to change the course of the child's illness
  - Decisional conflict related to treatment options and choice of setting for the child's final care

OUTCOME IDENTIFICATION AND PLANNING

- When planning care for children with a chronic or terminal illness, be sure the outcomes established are realistic.
- You probably cannot alter the course of a child's illness, but you can help parents cope with signs and symptoms of the illness, its progression, or the impending death.
- Parents who have not yet accepted the seriousness of their child's illness—who hold on to the hope that their child will eventually be cured or restored to full health—may make plans that the child cannot accomplish.
- Uncertainty regarding the course of an illness and prognosis can be a challenge, but hope can aid in the development of coping strategies to deal with the child's immediate and developing needs
- If parents believe they must shield the child or other family members from the truth, it can be hard for them to plan realistic outcomes, and they may need the support of healthcare providers.
- Focusing on hopeful but realistic outcomes, such as the child's comfort or their ability to independently use a wheelchair, can help a family move toward acceptance of the seriousness of the illness.

IMPLEMENTATION

- Parents serve as advocates for their children with chronic or terminal illnesses.
- . Challenges may include parental overprotection of the child with a long-term or terminal illness and decreased quality of parent–child communication, resulting in stunted autonomy development and low self-esteem of the child/adolescent
- Helping parents recognize their child's capabilities and encouraging collaborative problem solving and illness management are ways in which nurses can facilitate adjustment of families to the diagnosis and the ebbs and flows of chronic illness.
- Suggestions for helping children achieve developmental milestones are discussed for each age group in Chapters 29 through 33.
- Helping parents develop coping strategies and establish effective ways to manage the complex medical needs of the child, maintain quality care despite the burdens of care, and maintain a balanced life that includes self-care are other important issues to consider.

OUTCOME EVALUATION

- Children with a long-term or terminal illness need periodic follow-up care because children's needs across the life span change
- Examples:
- Parents state realistic plans for their child regarding school placement.
- Parents state they have been able to deal with their grief over their child's diagnosis to maintain near-normal family functioning.
- Parents state they are able to cope with present stressors and can specify coping strategies they utilize.
- the child states awareness that illness is chronic (long term) but thinks of themselves as a person who will be able to accomplish many things in life.

THE CHILD WITH A LONG-TERM ILLNESS

- Families have different resources, and everyone reacts to situations differently.
- Children with a long-term illness and their family need to be assessed individually for the potential to cope with the illness and to provide necessary care.
- Through such an assessment, appropriate interventions to help the family adapt can be started early in the course of the illness

THE PARENT'S ADJUSTMENT

- Children need support in adjusting to stressful events and parents can serve as role models while providing the emotional and instrumental support the child needs.
- An assessment begins with an examination of the parents' response to the stress of a long-term illness
- Chronic health conditions affect the entire family.
- Nurses should support each family member as their individual needs may vary at every stage of grieving.
- The nurse should seek to:
  - Answer the family's questions or seek assistance from the medical team in answering their questions.
  - Identify and address the needs of the family and child.
  - Assist the family in meeting the physical needs of the child, including pain relief and symptom management.
  - Assist the family with advanced care planning, including the inevitable effect on the physical and mental health of parents, siblings, and extended family.
  - Identify and assist with psychological and spiritual support.
  - Assist with care coordination appropriate to each stage of illness.

GRIEF RESPONSE

- Grief is an emotional response to loss.
- Bereaved parents' experience of grief that follows the diagnosis of a terminal illness and looking ahead to the inevitable challenges their child will experience is a complex set of emotions.
- Parents who lose a child who had a long-term illness have to come to accept the disruption of the natural course of family life, unique parent–child bond, while struggling to make sense of the loss
- The five-stage model of grief conceptualized by Kübler-Ross (1969) describes this emotional process, which includes denial, anger, bargaining, depression, and acceptance

TABLE 56.1 THE STAGES OF GRIEF

Stage	Parents' Reaction	Description
1	Denial	Parents have difficulty realizing what has occurred. They ask, "How could this have happened?"
2	Anger	Parents react to the injustice of being singled out this way. They say, "It isn't fair this is happening."
3	Bargaining	Parents attempt to work out a "deal" to buy their way out of the situation. They say, "If my child gets well, I'll devote the rest of my life to doing good."
4	Depression	Parents begin to face what is happening. They feel sad and unprotected.
5	Acceptance	Acceptance is being able to say, "Yes, this is happening, I will have to deal with it." With a child who has a long-term illness, parents may never reach this stage but will always remain in the chronic sorrow of the depression stage.

FACTORS INFLUENCING PARENTAL ADJUSTMENT

- The degree of illness
- The onset of the illness
- The effect of parental experience
- The availability of support
- Life events

THE CHILD'S ADJUSTMENT

- A child's reaction to being challenged physically or cognitively or to having a long-term illness is strongly influenced by the family's reaction to the illness
- Inclusion in school and social activities, acceptance by peers and support people, and the ability to function as normally as possible help the child adjust
- Iinfluenced by personal attitude and temperament, self-concept, age and development, understanding of the condition, and degree of the disorder.
- Degree of fatigue that a child experiences is instrumental in how well they adjust to a chronic illness because chronic fatigue can play a large role in making even simple tasks seem monumental
- Nursing interventions to help a child better adjust to a chronic condition include encouraging optimal growth and development (see Chapters 29 through 33), promoting self-care activities, enhancing self-esteem, preventing social isolation, providing health education, preventing fatigue, and supporting the child and family to identify strategies to adapt to living with the child's condition.

SIBLING'S ADJUSTMENT

- Siblings' reactions to a child with a long-term or terminal illness are influenced by individual circumstances; however, their reaction is most profoundly affected by the reactions and perceptions of the parents
- Without counseling, siblings may react with jealousy, anger, hostility, resentment, competition, guilt, or withdrawal.
- At any one time, the siblings may experience a mixture of feelings. For example, when parents must take their child for a medical procedure during the time when the sibling has an important event, the sibling may feel both resentment that the parents missed the event and sadness that the sibling's illness may prevent them from participating in activities such as sports

THE NURSE AND THE CHILD WHO IS ILL

- Can be stressful for nurses
- Nurses help the parents and child with a long-term illness review specific aspects of the child's condition and the possible complications that could occur
- Over a period of years, parents become experts in the care of a child with a particular condition
- When young children are receiving care at an ambulatory care setting or are admitted to a hospital for care, review their routines with the parents so that you can continue to care for their child in the same way

DEVELOPMENTAL TASKS

- Children with long-term illnesses often do not meet developmental milestones on schedule because achieving developmental tasks takes practice.
- When helping parents teach a new skill to a child with physical or cognitive challenges, such as toilet training or using a spoon, help them break the task down into its component parts (e.g., reach for the spoon, grasp it, push it under the selected food, and lift it toward the mouth).
- Breaking a task down into steps in this way allows parents to appreciate that they are asking their child to learn a task that, although it looks easy, actually encompasses 20 or more coordinated motions.
- Helping them learn this technique enables them to be patient when teaching their child additional tasks in the future.

EDUCATION

- Children with long-term illnesses often need special education programs or, at a minimum, individualized instruction to succeed in school.
- Most children benefit from preschool programs because these programs encourage social, emotional, and cognitive development.
- Children with a long-term illness may have absences because of routine healthcare visits and exacerbations of their illness.
- This can result in failure to meet educational success markers and may jeopardize academic progression.
- Federal law requires schools who receive federal funding to provide comprehensive programs
- Supporting children with additional health and developmental needs is a complex endeavor requiring a multidisciplinary approach

HOME CARE

- Many children with long-term illnesses require hospitalization related to their illness and may require home care nursing services
- Children with HMV require many services outside of the hospital setting such as home care, durable medical equipment, and therapy services
- Nursing shortages in home healthcare can have a significant impact on the lengths of hospital stay and the quality of care provided at home after discharge

THE CHILD WHO IS TERMINALLY ILL

- Death of a child may come unexpectedly to some families, such as death by suicide or when a child dies in a motor vehicle accident or random shooting.
- Parents who lose their child suddenly are provided no time to prepare emotionally.
- Parents of children with life-limiting illnesses expect the ultimate loss of their child but may still be caught off guard
- Nurses can be an integral part of this planning.
- Loss is a difficult experience for anyone; the death of a child may be especially hard for the caregivers as well as the care provider team.
- Nurses experience moral distress while caring for children who are terminally ill
- These feelings of distress linger after the initial incident and this residual distress can have an impact on the nurse's well-being, job satisfaction, as well as child and parental outcomes
- It is imperative for nurses to work through their own feelings of grief so they can find the strength to care for the child and support the parents.

PARENTAL GRIEF RESPONSES

- Many factors can influence a family's response to learning that their child has a terminal illness.
- Each family responds in a unique way.
- Being aware of the usual grief response that occurs when a child is diagnosed with a terminal illness helps in recognizing the grief response and supporting a family through this very difficult period
- The view of death and the manner in which people express grief differ across cultures and between individuals
- The way a child's body is prepared after death may differ.
- Being aware that grief is expressed differently and people have different expectations for final care enables a deeper understanding of parents' concerns and reactions during a child's terminal illness and at the time of death.
- Have a discussion with the parents or promote such a discussion prior to the death of the child, so the parents' desires are known and carried out

STAGE 1: DENIAL

- A parent's first response to the diagnosis of a terminal illness or long-term illness that may lead to a premature death in a child is denial
- Although people are aware that children die, most parents focus on finding a cure or encountering minimal challenges.
- When they realize they might lose their child, they respond with disbelief
- Can be influenced by relationship with healthcare providers
- You may find parent's downplaying their child's illness
- Denial is a temporary and necessary step on the way to acceptance, and individuals will move through this stage at different speeds

STAGE 2: ANGER

- Parents can be expected to enter a stage of anger—a change from “Surely not me” to “It’s not right this is happening to me.”
- When parents are angry about a diagnosis, they may be unable to direct their anger appropriately.
- Parents may be angry and argumentative toward one another or may become impatient with each other.
- They may direct their anger toward you
- Be certain that your first reaction is not to be angry in response. A more therapeutic response is to accept the parents’ explicit emotions and anger with empathy and acceptance.

STAGE 3: BARGAINING

- Although not everyone progresses through grief in a linear fashion, bargaining often follows anger.
- This is a time when parents try to correct or change the course of illness by bargaining.
- They vow to be better people or to function in a different way in exchange for their child's life.
- When parents realize that bargaining is ineffective, it brings them to a low point because they are let down not only by healthcare providers but also by the superior power with whom they tried to bargain.
- They may need more support after bargaining fails than at any other point

STAGE 4: DEPRESSION

- When parents have passed through the stages of denial, anger, and bargaining, the awareness of the reality of this diagnosis will usher them into the stage of depression.
- Frequent crying or projecting an overall sad demeanor is the most common sign of this stage.
- During this stage, parents need to think about preparing other children in the family for the challenges facing the child with the illness. If the child is hospitalized and siblings have not been allowed to visit, siblings may misinterpret the hospital experiences of the child and the reasons why visitation may be restricted. You may need to advocate for sibling visitation to overcome this
- These children need reassurance that the sibling's death is not a result of their interactions, but an unfortunate result of the child's illness. Assure them that they need not feel guilty.

STAGE 5: ACCEPTANCE

- The acceptance stage of the grief process is resolution that the child has an illness that may be fatal.
- Some parents do not reach this stage until the time of the child's death; working through grief will need to continue for years after a child's loss.

CHILDREN'S REACTIONS TOWARDS IMPENDING DEATH

- Children’s reactions to death are strongly influenced by their developmental stage, previous experiences, and the family’s attitude toward death.
- For instance, death can be a new or frightening phenomenon if a child has never had a pet die, was forbidden from visiting a dying relative, or was discouraged from discussing the death of a loved one.
- Children’s reactions to death are influenced by their stage of development and cognitive ability

INFANTS AND TODDLERS

- Infants and toddlers are certainly too young to appreciate the permanence of their death.
- If their caregiver dies, they experience a deep loss and a void in their life.
- If such a loss interferes with the development of a sense of trust its implications for the child's ability to achieve warm, close relationships could last a lifetime.

PRESCHOOLERS

- Preschoolers cannot always differentiate what is real from what is make-believe play.
- They envision death as temporary and therefore may not fear death like adults do.
- This view of death is sometimes interpreted as unfeeling. For example, the first response of a child who is told a sibling has just been killed in a motor vehicle accident might be to ask if they can have the sibling's cell phone.
- This happens because the child thinks of the sibling as being gone for only a short time, making this a chance to take advantage of their belonging.
- The understanding of death as temporary may be strengthened by fictional depictions of death in children's animation or stories in which sometimes characters die and come back to life.
- The separation anxiety preschoolers experience can complicate their response to death.
- When confronted with their own death, their main concern may be separation from their parents.
- Constant reassurances and having a caregiver present with the child can mitigate these fears.

SCHOOL-AGED CHILDREN

- School-aged children begin to have additional experiences with death, so their knowledge of its finality increases
- Children near 8 or 9 years of age begin to appreciate that death is permanent
- Most school-aged children are aware of what is happening to them when their disorder has a fatal prognosis.
- Unfortunately, they may learn of this prognosis from other children on the unit or at school, from their parents' unusual responses to questions, or from overhearing conversations about reports or physical findings
- Dying can be viewed as another new experience, which they are able to cope with as long as they know they will have someone with them. If the parents are unable to relate to a child because of their grief, a nurse may need to step in and assist
- Many children associate death with sleep, so they may be afraid to fall asleep without someone near them and will need to have you sit with them while they fall asleep.

ADOLESCENTS

- The capacity to think in abstract terms grows and an adolescent has a realistic perspective on death.
- However, adolescents have an unrealistic view of their own mortality and see themselves as invincible.
- Risky behaviors at this stage of development reflect this sense of invincibility.
- They may deny symptoms that reveal their condition is worsening for longer than you would expect because they believe it is impossible that anything serious could be happening to them.
- They appreciate time for reflecting on their contribution to their family or community while their own death is imminent.
- Continuing to participate in favorite activities helps them maintain a sense of control.

ENVIRONMENT FOR DEATH

- The Hospital
  - Complex care may necessitate the child stay in the hospital
  - Ensure visiting hours are adequately extended for family
  - Should be given choice to be present in resuscitation measures and have support person
- Hospice
  - Designed to be "home-like"
  - Unlimited visitation

PREP FOR A NURSING ROLE WITH DYING CHILDREN AND THEIR FAMILY

- End-of-life care for children can be an emotionally distressing experience for healthcare providers as well as family members.
- Although it is best that nursing assignments be consistent so a child has meaningful support, there is a point at which a healthcare provider may need a respite from caring for the child or providing support for the parent
- Before you can offer support to children in any circumstance, be aware of your own reactions and feelings.
- Self-Awareness: Before you can offer support to children in any circumstance, be aware of your own reactions and feelings
- Grief: Nurses become intensely involved in the therapeutic relationship with children and their families

CARING FOR THE CHILD WHO IS DYING

- A child may live for days, weeks, or even months at the end-of-life stage.
- Attentive physical and emotional care is essential for a child to maintain a sense of security and positive self-esteem during this time.
- It is also essential to assess if parents are coping well, especially if this period is prolonged.
- Frequent and substantive communication is a major part of providing end-of-life care.
- Children, like their parents, need the opportunity to talk about their fears and feelings about death.
- Practicing good communication skills when providing any care such as when administering pain medication, starting intravenous lines, or providing basic comfort measures help to establish a trusting relationship with the child, making the child feel more comfortable about openly sharing their feelings
- (Fig. 56.4). Box 56.6 provides some specific guidelines about communicating with a child who is dying.

BOX 56.6

Nursing Care Planning to Establish Therapeutic Relationships

MEASURES TO HELP COMMUNICATION WITH CHILDREN WHO ARE TERMINALLY ILL

1. *Continue active conversation*: Children who are dying still need to be talked to and engaged in activity to the best of their ability.
2. *Use moments of silence therapeutically*: Such moments occur normally. Do not feel obligated to chatter to fill quiet intervals. Silence allows for processing and reflection.
3. *Use the words "death" and "dying" as appropriate in conversation*: Trying to avoid a word makes interchanges awkward. The word "death" or "dying" can be mentioned without hesitation in sentences as appropriate.
4. *Preserve dying children's defenses*: If they are using denial or bargaining, do not try to push them to the next step of grieving by confrontation. Children will move on to the next step when they are psychologically ready.
5. *Terminally ill children may feel more vulnerable at night*: They may be comforted by having someone sit beside them until they fall asleep.
6. *Be supportive*: A statement such as "All of us are dying" is true but not helpful. A supportive statement such as "This must be hard for you" is more appropriate.
7. *Be aware that not all people's beliefs are the same as yours*: Statements such as "God works in mysterious ways" may explain death for one person but not for another. People grieve and react to grief in many different ways.

THE ONSET OF DEATH

- Be certain to keep skin surfaces from rubbing against one another by using supportive pillows and good positioning.
- Keep the skin free of urine or feces from incontinence to prevent painful ulcers.
- Urine output will decrease significantly in this stage.
- Assess for indications of pain (e.g., thrashing, moaning) and provide relief with appropriate comfort measures

DOCUMENTATION OF DEATH

- Defining when death occurs is controversial and involves both legal and ethical issues.
- As a child nears death, ethics committees are beneficial to assist families in decision making.
- Signs of death in a child not receiving ventilator or mechanical assistance are the same as in adults:
  - Absence of respirations
  - No audible heart sounds by stethoscope
  - No pulse by palpation
  - No apparent blood pressure
  - Absence of body movement or reflexes
  - Dilated, fixed pupils
  - Death is officially determined by a lack of receptivity and responsivity, no spontaneous muscular movement or breath, no reflex response, and a flat electroencephalogram.

ORGAN DONATION

- Primary healthcare provider or a specifically designated transplantation team may ask parents, prior to their child's death, to grant permission for body organs to be transplanted into other children after death occurs
- This is particularly important for kidney transplants because it is difficult to transplant adult-size organs into children.
- If parents decide to allow organ donation, document this information on the child's electronic record and alert the primary care provider about the decision.
- When death does occur, the child's body is maintained on life support to ensure that the organ remains perfused until removal.
- The donation of body organs is not something that all parents will consent to, but it can assist parents who choose this option to accept their child's death because it allows them to feel that their child has helped another person live.
- Donating their child's organs is not a practice for everyone and particularly in those who may be skeptical of healthcare providers

AFTERCARE

- Check with family members to see if they want to spend time with the child or if there are any activities they want to complete before the body is transported to the morgue and funeral home.
- Some parents need time for closure. Some people have special prayers or traditions and rituals they need to follow; others want to say a final, private goodbye.
- Check that the child's bed and room is orderly; particularly if the death was following an unsuccessful resuscitation attempt.
- Remain in the room with the family in case they need your support, but be unobtrusive.
- Some parents fear touching a child's body after death,
- Parents' externalized expression of grief may vary.
- When nurses cry in response to a child's death, it is not unprofessional. A parent's warmest memory of a hospital experience may be that a nurse cried while saying goodbye to their child—the implication being the child made an important impact on people other than the family

AUTOPSY PERMISSION

- State laws vary, but generally, if a child's death is a result of homicide, suicide, death within 24 hours of a hospital admission, suspected harmful death, or death in an institution or home where the child was not under the care of a primary healthcare provider, law requires an autopsy and parents have no input regarding whether it should be performed.
- In other instances, it would be helpful to medical progress or to research if an autopsy could be done.
- In these instances, parents must consent. Parents may refuse to allow an autopsy for a child because of spiritual or religious beliefs.
- Autopsies advance medical science, so they should be done if possible; however, parents have every right to refuse permission without being made to feel guilty for their actions, so you may need to advocate for them.